

UID: 71EC1611-E8A2-4692-8C6A-9FF1DEC0AE85
TCGA-L6-A4ET-01A-PR

Redacted

Gender: [REDACTED]

Birth Date: [REDACTED]

Admit Protocol: [REDACTED]



Final Results

Surgical Pathology

Final

CASE NUMBER: [REDACTED]

DIAGNOSIS:

1. Thyroid, right lobe, total thyroidectomy:
 - Papillary thyroid carcinoma, follicular variant, (at least 6cm greatest dimension).
 - Tumor is present focally at black inked anterior margin. (See CAP checklist below.)
 - Extensive post-Fine Needle Aspiration artifact.
2. Thyroid, right, inferior, nodule, total thyroidectomy:
Adenomatoid nodule; no tumor seen.
3. Thyroid, left lobe, total thyroidectomy: Adenomatoid nodules; no tumor seen.
4. Thyroid, left, tubercle, total thyroidectomy: Benign thyroid tissue; no tumor seen.

NOTE:

Thyroid carcinoma summary
Procedure: Total thyroidectomy

Received: Fresh

Specimen Integrity: Intact

Specimen Size:

Right lobe and isthmus: 7cm x 3.9 cm x 2.1 cm

Left lobe and isthmus: 5.1cm x 2.1cm x 1.2 cm

Specimen Weight: 55.5g

Tumor Focality: Unifocal

Dominant Tumor:

Tumor Laterality: Right lobe

Tumor Size:

Dimensions: 2.5 x 3cm x 6cm*

*Greatest dimension: at least 6cm. Exact measurement is not possible since specimen was procured for research tissue in the OR by surgeon and post-procurement, the specimen measured 7cm in greatest dimension. The specimen was entirely submitted and based on the presence of tumor in most of the sections, we estimate the greatest dimension of the tumor to be at least 6cm.

Histologic Type: Papillary carcinoma

Variant: Classical

Architecture: Follicular

Cytomorphology: Classical

ICD-O-3

carcinoma, papillary, follicular variant
8340/3

Site: Thyroid, NOS C73.9

lw
9/24/12

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Histologic Grade: Moderately differentiated

Margins: Tumor is present focally at black inked anterior margin (sections 1I and 1T).

Tumor Capsule: Encapsulated

Tumor Capsular Invasion: Not identified

Lymph-Vascular Invasion: Not identified

Perineural Invasion: Not identified

Extrathyroidal Extension: Cannot be assessed

Pathologic Staging:

Primary Tumor (pT)

pT3: Tumor more than 4 cm limited to thyroid or any tumor with minimal extrathyroid extension (eg, extension to sternothyroid muscle or perithyroid soft tissues)

Regional Lymph Nodes (pN)#

pNX: Cannot be assessed

Lymph Node, Extranodal Extension
IndeterminateDistant Metastasis (pM)
Not applicableAdditional Pathologic Findings:
Adenomatoid nodules

Immunoperoxidase and in-situ hybridization tests performed here and used for diagnosis were developed and their performance characteristics determined by the Laboratory of Pathology. They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has also determined that such clearance or approval is not necessary. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing.

CLINICAL INFORMATION: Brief Clinical History: Right thyroid mass with FNA showing papillary cells Specimen Taken For Protocol: 01 - Yes Allocate Order to

PROCEDURE: Pre-Operative Diagnosis: Right thyroid mass Post-Operative Diagnosis:

Confidential Patient Information. Unauthorized disclosure is prohibited by the Federal Privacy Act of 1974.

Requested By: [REDACTED] (Other)

Do not file in Medical Record

Gender: [REDACTED]

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Age: [REDACTED]

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Same Operative Findings: Enlarged right lobe, small left lobe, significant adhesion

SPECIMENS SUBMITTED: 1. THYROID, Right lobe
2. SOFT TISSUE, NOS, Right inferior nodule
3. THYROID, Left lobe
4. SOFT TISSUE, NOS, Left tubercle

GROSS DESCRIPTION: Received are 4 freshly submitted specimens each in containers labeled with the patient's name, medical record number, and further specified as follows:

1. "Right thyroid lobe: single stitch - isthmus, double stitch - superior. The specimen is red-tan firm thyroid tissue mass measuring 7 cm x 3.9 cm x 2.1 cm and weighs approximately 52g. The outer surface is rough. The specimen is oriented by Dr. [REDACTED] double stitch superior, single stitch hilum. The lateral surface of the specimen is cut by Dr. [REDACTED] in the OR showing hard white tan homogeneous nodule measuring 2 cm x 2 cm. Gross photographs are taken. 30% of the nodule is procured for Dr. [REDACTED] and [REDACTED] by Dr. [REDACTED]. The remainder of the specimen is placed into formalin and submitted to Pathology permanent. Procurement was documented on [REDACTED]. Post procurement, the anterior is inked black, and the posterior is inked green. The specimen is received in Surgical Pathology and matches the above description. The specimen is received in Surgical Pathology and matches the above description. The specimen is serially sectioned from superior to inferior and entirely submitted in white cassettes. There appears to be hard white calcifications in the thyroid tissue. [REDACTED] corresponds to the nodular area that was procured.

2. "Right inferior nodule". The specimen consists of tan-white nodular, soft tissue measuring approximately 2.7 cm x 0.8 x 0.6 cm. The specimen is sectioned, staples removed, and entirely submitted in white cassettes labeled [REDACTED]. Sectioning, there appears to be 2 nodules measuring approximately 0.8 cm and 0.7 cm. One of the nodules, the smaller node, appears to have a cystic lesion in the center of it.

3. "Left thyroid lobe single stitch: isthmus, double stitch: superior" is an oriented partial thyroidectomy specimen consisting of the left thyroid lobe and minimal if any portion of isthmus, overall 5.1 x 2.1 x 1.2 cm and 3.5 grams. The external surface is rough and irregular with focal cautery artifact. There is a double suture denoting the superior left pole and a single suture denoting the isthmus. Additional sutures are evident on the posterior aspect where Dr. [REDACTED] state that the left tubercle was transected (see specimen #4). In the OR, Dr. [REDACTED] made one cut at the superior medial aspect revealing red-pink fleshy normal-appearing parenchyma. Gross

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photographs are taken. Approximately 0.8 x 0.5 x 0.4 cm of the normal-appearing thyroid parenchyma is procured for research with equal halves provided to Dr. [REDACTED] and Dr. [REDACTED]. The anterior surface is inked red, posterior surface inked black, and the isthmus is inked green. Left thyroid lobe itself appears to be comprised of at least 4 soft nodular tissues intermixed with normal parenchyma. The nodules range from 0.6 to 1.1 cm. The remainder of the specimen is placed into formalin and submitted to Pathology for permanent processing. The procurement was performed by [REDACTED] and [REDACTED] in the OR. The specimen is received in Surgical Pathology and matches the above description. On sectioning, two nodules are seen measuring approximately 0.6 cm and 0.7 cm in greatest dimensions. The specimen is sectioned and submitted entirely from the inferior to the superior in white cassettes labeled [REDACTED] and 3C: nodules.

4. "Left tubercle". The specimen consists of a nodular tan soft tissue fragment measuring approximately 2.1 cm x 0.8 cm x 0.3 cm. The specimen is sectioned and entirely submitted in white cassette labeled [REDACTED].

Gross Description dictated by [REDACTED]
gross description by D. [REDACTED]

Additional

No consultants

Accessioned:

Final Report Signed Out:

<Resident Signature>

<Sign Out Dr. Signature>

≥ 99% follicular - per TSS.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case Is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials		
Date Reviewed		

9/18/17

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Requested By: [REDACTED] (Other)

Do not file in Medical Record